

UK HEALTHCARE · PROBLEM → SOLUTION

One letter, one loop, one coded record

The gap in UK care data, and the flow that closes it — with the diagrams.

Slides 2–3: the problem (dark) · Slides 4–6: the solution (light) · Independent project, not affiliated with the NHS

THE PROBLEM · 1 OF 2

Four records, one patient, no join

GP practice

Independent contractor. The lifelong record sits in EMIS Web or SystmOne.

Hospital trust

Its own electronic patient record — separate system, separate login.

Pharmacy

Its own dispensing record, often unconnected to the practice.

National

NHS number, Spine, GP Connect — plumbing, not the record.

So what joins them is **not a database**. It is correspondence: discharge summaries and clinic letters, arriving as documents.

A document is not a data point



A person must read it and code it — until then the record does not know

Coding lag: the fact is on paper, not in the record being searched.

Recall misses: a recall queries codes; free text is invisible to it.

Duplication: the same datum typed again into a second system.

THE SOLUTION · 1 OF 3

Close the gap with four steps

1

Read

Letter or PDF in; diagnosis, medicine, action and dates out.

2

Propose

A SNOMED CT code per fact, with a confidence — not a decision.

3

Confirm

A clinician accepts, edits or rejects. Human, before the record.

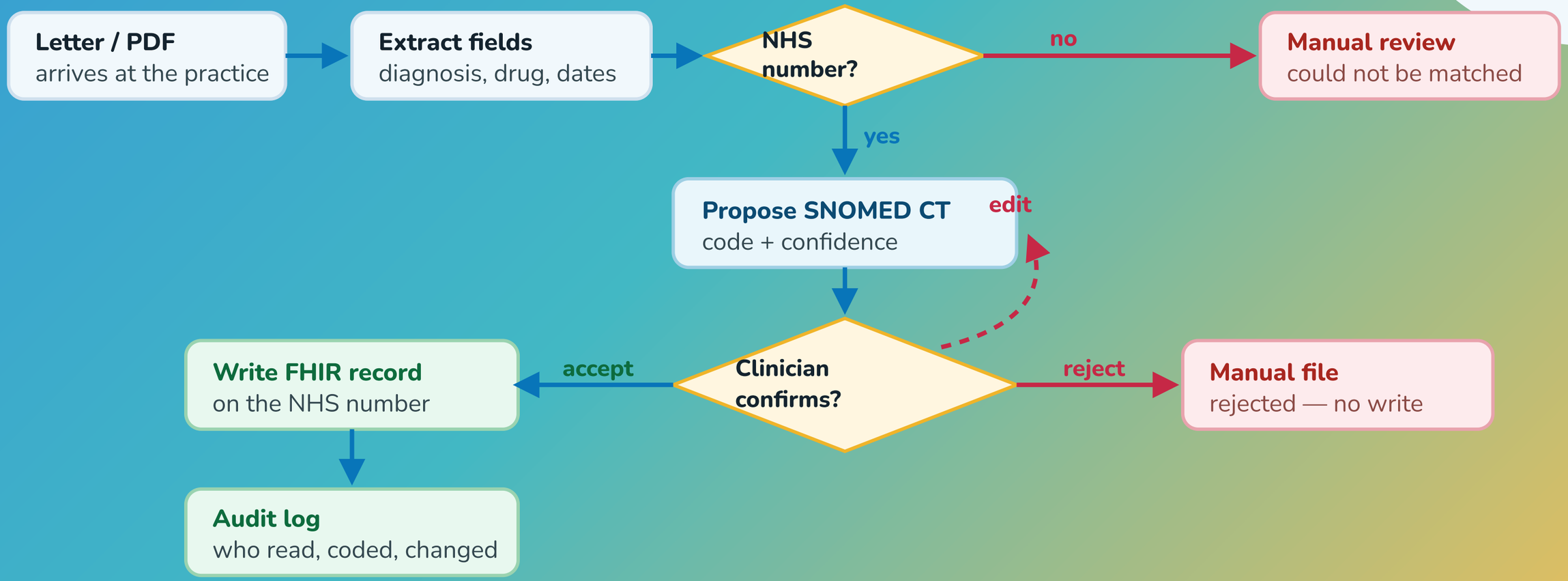
4

File

FHIR R4 records on the NHS number, with an audit trail.

The machine does the reading. The clinician keeps the decision. Nothing is written silently.

What happens to one letter



THE SOLUTION · 3 OF 3

Standards, limits, and where to look

STANDARD	USED FOR
NHS number	Linking the patient
SNOMED CT	The proposed code
dm+d	Medicine identity
FHIR R4	Records as data

No autonomous diagnosis. A clinician confirms every code.

No shared pool. One practice cannot see another's patients.

No compliance claim. DTAC, DSPT and DCB0129 come first.

See the loop in 30 seconds — six clickable screens, sample data.

19p1362.github.io/portfolio/healthbridge

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